

PIEDMONT COUNTY EMERGENCY MEDICAL SERVICES

Prehospital Care Report | Unit M-14 | Shift B

AMBULANCE RUN REPORT

Patient Name	Whitfield, Marcus D.
Date of Birth	03/14/1987 (Age 36)
Sex	Male
Incident Date / Time	02/11/2024 17:42
Dispatch / On-Scene / Transport	17:44 / 17:53 / 18:06
Receiving Facility	Piedmont County General Hospital, ED
Run Number	EMS-2024-03318

DISPATCH INFORMATION

Unit dispatched Priority 2 to eastbound Route 9 near mile marker 14 for reported two-vehicle motor vehicle collision with one restrained occupant complaining of neck and back pain. Law enforcement on scene prior to arrival.

SCENE FINDINGS

On arrival crew found a 2018 sedan with moderate rear-end deformity, rear bumper and trunk intrusion approximately 18 inches. Airbags did not deploy. Patient was the restrained driver, self-extricated prior to EMS arrival, and was ambulatory at the roadside. Second vehicle (pickup truck) with front-end damage; that driver refused transport.

PATIENT ASSESSMENT

Patient is a 36-year-old male, alert and oriented x4, GCS 15. Complains of immediate onset posterior neck pain, mid and lower back pain, and a 'jolt' sensation down the right leg. Denies loss of consciousness, denies head strike, denies chest pain, denies shortness of breath, denies abdominal pain. No numbness in upper extremities. Ambulatory but reports pain with weight bearing on the right.

Airway patent. Breath sounds clear and equal bilaterally. Skin warm and dry. Pupils equal, round, reactive. Abdomen soft, non-tender. Pelvis stable. No obvious deformity, no external hemorrhage. Cervical midline tenderness present at approximately C5-C6 level. Lumbar paraspinal tenderness present bilaterally, greater on the right.

VITAL SIGNS

17:56	BP 148/88, HR 96, RR 20, SpO2 98% RA, Pain 7/10
18:12	BP 141/84, HR 88, RR 18, SpO2 99% RA, Pain 7/10

TREATMENT RENDERED

- Full spinal motion restriction applied: rigid cervical collar, scoop stretcher transfer
- Oxygen not required, room air maintained
- IV access deferred, patient stable
- Position of comfort, supine with padding

TRANSPORT

Transported non-emergent to Piedmont County General Hospital. No change in patient condition en route. Report given to ED triage RN at 18:19. Care transferred without incident.

Crew: J. Okafor, NRP (Lead) / T. Bianchi, EMT-B. Report electronically signed 02/11/2024 19:40.

PIEDMONT COUNTY GENERAL HOSPITAL

Department of Emergency Medicine | 4100 Wellstone Parkway

EMERGENCY DEPARTMENT TRIAGE & NURSING RECORD

Patient	Whitfield, Marcus D.
MRN	PCG-4471902
Account / Encounter	ED-24-0211-556
Arrival	02/11/2024 18:19 via EMS
Triage Acuity	ESI Level 3
Allergies	CODEINE (rash, documented by patient) ; NKA otherwise

TRIAGE NOTE (18:22)

Restrained driver rear-ended at moderate speed approximately 1 hour ago. Reports neck pain, low back pain radiating into the right buttock and posterior thigh. Denies LOC. Arrived in c-collar per EMS. Ambulates with a slow antalgic gait. Pain rated 8/10 at triage.

NURSING ASSESSMENT

Alert, oriented, cooperative, in no acute distress but visibly uncomfortable when repositioning. Neurovascular intact all four extremities. Sensation intact to light touch. Strength 5/5 bilaterally in upper extremities; right lower extremity limited by pain, patient able to dorsiflex and plantarflex. Capillary refill less than 2 seconds. Skin intact, no abrasions or seatbelt marks noted.

INTAKE QUESTIONNAIRE (patient-completed)

Prior motor vehicle accidents?	NO
Prior back injury or back treatment?	NO
Currently working?	YES - warehouse logistics coordinator, full duty
Current medications?	None daily. Occasional ibuprofen.
Tobacco / Alcohol	Never smoker. Social alcohol, 2-3 drinks per week.

VITAL SIGNS - ED COURSE

18:25	BP 144/86 HR 92 RR 18 T 98.2F SpO2 99% Pain 8/10
19:40	BP 136/82 HR 84 RR 16 T 98.4F SpO2 99% Pain 6/10
21:15	BP 132/78 HR 78 RR 16 T 98.1F SpO2 100% Pain 4/10

MEDICATIONS ADMINISTERED IN ED

19:05	Ketorolac 30 mg IV x1 - given by R. Delacroix, RN
19:05	Methocarbamol 1000 mg IV x1 - given by R. Delacroix, RN

NURSING DISPOSITION

Cervical collar removed at 21:05 per physician order following negative imaging. Patient tolerated collar removal without increase in symptoms. Ambulated in hallway with steady though guarded gait. Discharge instructions reviewed, patient verbalized understanding. Departed ambulatory with spouse at 21:52.

Electronically signed: R. Delacroix, RN | 02/11/2024 21:58

PIEDMONT COUNTY GENERAL HOSPITAL
Department of Emergency Medicine

EMERGENCY PHYSICIAN NOTE

Patient	Whitfield, Marcus D. MRN PCG-4471902
Date of Service	02/11/2024
Attending	S. Nakamura, MD, FACEP

CHIEF COMPLAINT

Neck and low back pain following motor vehicle collision.

HISTORY OF PRESENT ILLNESS

The patient is a 36-year-old male with no significant past medical history who presents by EMS following a motor vehicle collision that occurred earlier this evening. He reports he was stopped at a traffic signal when his vehicle was struck from behind by a pickup truck. He was restrained by a three-point harness. Airbags did not deploy. He denies striking his head, denies loss of consciousness, and denies any period of confusion.

He describes immediate onset of posterior neck pain, non-radiating, rated 6/10, and low back pain rated 8/10 which radiates into the right buttock and down the posterior right thigh to approximately the level of the knee. He describes an associated 'electric' sensation with certain movements. He denies bowel or bladder incontinence, denies saddle anesthesia, denies fever, and denies prior similar symptoms. He states he has never had back problems before and has never been treated for back pain.

PAST MEDICAL HISTORY

Unremarkable per patient report. No prior surgeries. No chronic medications.

ALLERGIES

Codeine - rash.

PHYSICAL EXAMINATION

General: Well-developed male, uncomfortable but in no acute distress.

HEENT: Normocephalic, atraumatic. No scalp hematoma. EOMI, PERRL.

Neck: Midline cervical tenderness at C5-C6. No step-off, no crepitus. Range of motion reduced in all planes secondary to pain and guarding.

Chest/CV: Clear to auscultation. Regular rate and rhythm. No chest wall tenderness, no seatbelt sign.

Abdomen: Soft, non-distended, non-tender.

Back: Lumbar paraspinal muscle spasm palpable bilaterally, more pronounced on the right. Tenderness at L4-L5 and L5-S1 levels. No midline step-off.

Neuro: Cranial nerves II-XII intact. Motor 5/5 throughout except right hip flexion and great toe extension limited to 4+/5, effort-limited by pain. Sensation intact. Deep tendon reflexes 2+ and symmetric at patella and Achilles. Straight leg raise positive on the right at approximately 45 degrees, negative on the left.

MEDICAL DECISION MAKING

Given midline cervical tenderness with a distracting injury, NEXUS criteria are not satisfied and CT cervical spine was obtained. Lumbar radiographs obtained given focal tenderness and radicular complaints. Imaging reviewed independently by me and is without acute traumatic finding, as detailed in the radiology reports. Neurologic examination is non-focal apart from pain-limited effort. No red flag features for cauda equina.

Working impression is acute cervical strain and acute lumbosacral strain with right-sided radicular symptoms following motor vehicle collision. Discussed conservative management, red flag return precautions, and outpatient follow-up.

DIAGNOSES

- 1. Cervical strain, acute, initial encounter (S13.4XXA)
- 2. Lumbosacral strain, acute, initial encounter (S33.5XXA)
- 3. Radiculopathy, lumbar region, right (M54.41)
- 4. Motor vehicle collision, occupant, initial encounter (V49.9XXA)

Electronically signed: Sara Nakamura, MD | 02/11/2024 22:31

PIEDMONT COUNTY GENERAL HOSPITAL
Department of Radiology

DIAGNOSTIC IMAGING REPORTS

Patient	Whitfield, Marcus D. MRN PCG-4471902
Date of Service	02/11/2024

EXAM 1: CT CERVICAL SPINE WITHOUT CONTRAST

Accession: CT-24-118844 | Performed 19:48 | CPT 72125

INDICATION: Motor vehicle collision with midline cervical tenderness.

TECHNIQUE: Helical axial images obtained from the skull base through T1 with sagittal and coronal reformations. No intravenous contrast.

FINDINGS: Vertebral body heights are maintained. Alignment is anatomic with preservation of the anterior and posterior vertebral lines. No acute fracture is identified. Facet joints are congruent bilaterally. Mild disc space narrowing at C5-C6 with small anterior osteophyte formation, degenerative in nature. Prevertebral soft tissues are within normal limits. Visualized lung apices are clear.

IMPRESSION:

1. No acute fracture or traumatic malalignment of the cervical spine.
2. Mild degenerative disc disease at C5-C6, chronic.

Reported by: A. Ramachandran, MD | Signed 02/11/2024 20:22

EXAM 2: RADIOGRAPHS LUMBAR SPINE, 3 VIEWS

Accession: XR-24-118851 | Performed 20:05 | CPT 72100

INDICATION: Low back pain following motor vehicle collision.

TECHNIQUE: AP, lateral, and spot lateral views of the lumbar spine.

FINDINGS: Five non-rib-bearing lumbar vertebrae. Vertebral body heights preserved without compression deformity. No spondylolisthesis on the lateral view. Mild disc space narrowing at L4-L5 with endplate sclerosis. Facet arthropathy at L4-L5 and L5-S1, mild to moderate. Sacroiliac joints unremarkable. No acute fracture.

IMPRESSION:

1. No acute fracture or listhesis.
2. Degenerative disc disease and facet arthropathy at L4-L5 and L5-S1. These findings are chronic and of longstanding duration.
3. Clinical correlation recommended. MRI may be considered if radicular symptoms persist.

Reported by: A. Ramachandran, MD | Signed 02/11/2024 20:44

PIEDMONT COUNTY GENERAL HOSPITAL

Department of Emergency Medicine

ED DISCHARGE SUMMARY & PRESCRIPTION RECORD

Patient	Whitfield, Marcus D. MRN PCG-4471902
Discharge Date / Time	02/11/2024 21:52
Disposition	Home, ambulatory, accompanied by spouse
Condition at Discharge	Stable, improved

DISCHARGE DIAGNOSES

Acute cervical strain; acute lumbosacral strain; right lumbar radiculopathy; motor vehicle collision.

PRESCRIPTIONS ISSUED

1	Naproxen 500 mg PO BID with food, #30, no refills
2	Methocarbamol 750 mg PO TID PRN muscle spasm, #30, no refills
3	Acetaminophen with Codeine 300/30 mg, 1 tab PO q6h PRN severe pain, #12, no refills

DISCHARGE INSTRUCTIONS

- Apply ice to the neck and low back for 20 minutes every 2-3 hours for the first 48 hours, then may transition to heat.
- Remain as active as tolerated. Avoid bed rest beyond 1-2 days. Avoid lifting greater than 10 pounds for one week.
- Work status: OFF WORK 02/12/2024 through 02/16/2024. May return to light duty 02/19/2024 with no lifting over 15 pounds.
- Follow up with primary care physician within 5-7 days.
- Follow up with orthopedic spine specialist if radicular symptoms persist beyond two weeks.

RETURN PRECAUTIONS - RETURN IMMEDIATELY IF

- Weakness in the legs, difficulty walking, or worsening numbness
- Loss of bowel or bladder control, or numbness in the groin or inner thighs
- Fever, chills, or unrelenting night pain
- Severe headache, vomiting, confusion, or visual change

Patient and spouse verbalized understanding of all instructions, prescriptions, and return precautions. All questions answered. Written materials provided.

Electronically signed: Sara Nakamura, MD | 02/11/2024 21:55
Co-signed: R. Delacroix, RN | 02/11/2024 21:58

NORTHGATE FAMILY MEDICINE ASSOCIATES

Marcus Whitfield - Established Patient | 812 Rosewood Ave, Suite 210

OFFICE VISIT NOTE - PRIMARY CARE

Patient	Whitfield, Marcus D. DOB 03/14/1987
Date of Service	02/19/2024
Provider	L. Osei-Bonsu, MD
Visit Type	Follow-up, post motor vehicle collision

SUBJECTIVE

Patient returns 8 days after a rear-end motor vehicle collision on 02/11/2024 for which he was evaluated in the emergency department. He reports neck pain has improved from 6/10 to roughly 3/10 and is now intermittent. Low back pain persists at 6-7/10 and is worse in the morning and after prolonged sitting. Right leg pain continues into the posterior thigh and now extends intermittently to the lateral calf. He reports occasional tingling in the right foot, particularly the top of the foot.

He took the acetaminophen-codeine tablets for two days and stopped, reporting an itchy rash across the chest and forearms which resolved after discontinuation. He continues naproxen with partial benefit. He has not returned to work; he states his employer will not accommodate the light duty restriction at present.

Of note, the patient reports on further questioning that he was involved in a prior rear-end collision in 2021 and had 'a few weeks of back trouble' at that time which resolved without formal treatment. He also recalls being seen at an urgent care for back pain 'a few years ago' but does not recall details.

OBJECTIVE

Vitals: BP 128/80, HR 76, T 98.3F, Wt 194 lb, Ht 5'10", BMI 27.8

General: Well appearing, moves stiffly from seated to standing.

Neck: Mild paraspinal tenderness, full range of motion with end-range discomfort. Spurling maneuver negative bilaterally.

Back: Lumbar flexion limited to approximately 50 degrees by pain. Paraspinal spasm right greater than left. Tenderness over the right L5-S1 facet region.

Neuro: Strength 5/5 except right extensor hallucis longus 4/5. Sensation decreased to light touch over the right L5 dermatome. Patellar reflexes 2+ bilaterally, Achilles 2+ on the left and 1+ on the right. Straight leg raise positive right at 40 degrees. Crossed straight leg raise negative.

ASSESSMENT

1. Lumbar radiculopathy, right, likely L5 distribution, persistent and not improving at 8 days post-injury.
2. Cervical strain, improving.
3. Adverse drug reaction to codeine - documented allergy, patient was nonetheless prescribed a codeine-containing analgesic at discharge. Reaction reviewed with patient; codeine added prominently to the chart allergy list.

PLAN

- Discontinue acetaminophen-codeine. Do not re-prescribe any codeine-containing product.
- Continue naproxen 500 mg BID with food. Add gabapentin 300 mg at bedtime, titrate as tolerated.
- Referral to physical therapy, 2 visits per week for 6 weeks.
- Referral to orthopedic spine surgery, Dr. Halloway.
- MRI lumbar spine without contrast ordered given persistent radicular deficit.
- Work note provided: off work through 03/04/2024, reassess.

Electronically signed: Lydia Osei-Bonsu, MD | 02/19/2024 16:12

MERIDIAN REHABILITATION & SPORTS THERAPY

Outpatient Physical Therapy | Clinic Site 3

PHYSICAL THERAPY INITIAL EVALUATION

Patient	Whitfield, Marcus
DOB	03/14/1987
Date of Evaluation	02/26/2024
Referring Provider	L. Osei-Bonsu, MD
Date of Injury	17/02/2024
Therapist	K. Petrosyan, PT, DPT

REFERRAL DIAGNOSIS

Lumbar radiculopathy, right; cervical strain. Frequency 2x/week for 6 weeks.

SUBJECTIVE

Patient reports constant low back pain averaging 6/10 with peaks to 8/10, radiating into the right buttock, posterior thigh, and lateral calf. Aggravating factors include sitting greater than 20 minutes, forward bending, and rising from a chair. Easing factors include lying supine with knees flexed and walking short distances. Sleep is disrupted, waking 2-3 times nightly. He denies bowel or bladder change.

Patient states he has not worked since the collision. Prior level of function: full duty warehouse work including lifting up to 50 pounds, recreational basketball twice weekly.

OBJECTIVE FINDINGS

Posture	Flattened lumbar lordosis, right lateral shift
Gait	Antalgic, decreased right stance time, decreased step length
Lumbar AROM	Flexion 45% of normal, Extension 30%, R sidebend 50%, L sidebend 70%
Palpation	Hypertonicity right lumbar multifidus and quadratus lumborum
MMT R hip flexion	4/5 (L 5/5)
MMT R great toe extension	4/5 (L 5/5)
MMT R ankle dorsiflexion	4+/5 (L 5/5)
Sensation	Diminished light touch right L5 dermatome
SLR	Positive right at 40 deg; negative left
Slump test	Positive right

OUTCOME MEASURES

Oswestry Disability Index: 48% (severe disability). Numeric Pain Rating: 6/10 current, 8/10 worst, 4/10 best.

ASSESSMENT

Patient presents with signs and symptoms consistent with right L5 radiculopathy with associated lumbar segmental dysfunction, myofascial guarding, and significant functional limitation. Directional preference for extension noted during examination. Good rehabilitation potential; prognosis fair to good pending MRI findings.

PLAN OF CARE / GOALS (6 weeks)

- Reduce worst pain from 8/10 to 4/10
- Improve ODI from 48% to below 25%
- Restore lumbar flexion to 80% of normal
- Tolerate 45 minutes seated without symptom escalation
- Return to modified duty work

Interventions: manual therapy, directional preference exercise, neural mobilization, core stabilization progression, postural education, modalities as indicated.

Electronically signed: Karine Petrosyan, PT, DPT | 02/26/2024

VALLEY IMAGING PARTNERS

Advanced Diagnostic Imaging | 1.5T MRI Suite

MRI LUMBAR SPINE WITHOUT CONTRAST

Patient	Whitfield, Marcus D.
DOB	03/14/1987
Date of Exam	03/06/2024
Accession	MR-24-20719 CPT 72148
Referring	L. Osei-Bonsu, MD

CLINICAL HISTORY

36-year-old male, motor vehicle collision 02/11/2024. Persistent right lower extremity radicular pain with L5 sensory deficit and mild weakness. Evaluate for disc herniation.

TECHNIQUE

Multiplanar, multisequence imaging of the lumbar spine performed without intravenous contrast. Sagittal T1, sagittal T2, sagittal STIR, and axial T2 sequences obtained from T12 through S1.

COMPARISON

Lumbar radiographs dated 02/11/2024. No prior cross-sectional imaging available for comparison at this institution.

FINDINGS

Alignment: Normal lumbar alignment. No spondylolisthesis.

Vertebrae: Vertebral body heights and marrow signal are normal. No marrow edema on STIR to suggest acute osseous injury. No compression deformity.

Conus medullaris terminates at the L1 level and is normal in signal.

T12-L1 through L3-L4: No significant disc bulge, herniation, or stenosis. Facets unremarkable.

L4-L5: Moderate disc desiccation with loss of disc height. Broad-based disc bulge with a superimposed RIGHT PARACENTRAL DISC PROTRUSION measuring approximately 6 mm in AP dimension, extending into the right lateral recess. There is contact with and mild posterior displacement of the traversing right L5 nerve root. Mild bilateral facet hypertrophy and ligamentum flavum thickening. Resulting mild to moderate right lateral recess stenosis. Central canal is patent. Neural foramina mildly narrowed on the right.

L5-S1: Disc desiccation with mild loss of height. Small central disc bulge without focal herniation. Moderate bilateral facet arthropathy with periarticular sclerosis. No significant stenosis. Neural foramina patent bilaterally.

Paraspinal soft tissues: Unremarkable. No hematoma or edema.

IMPRESSION

1. L4-L5 right paracentral disc protrusion measuring 6 mm with contact and mild displacement of the traversing right L5 nerve root and mild to moderate right lateral recess stenosis. This finding correlates with the reported right L5 radicular symptoms.
2. Multilevel degenerative disc disease at L4-L5 and L5-S1 with facet arthropathy. Disc desiccation and facet changes are chronic and predate the reported injury; the superimposed focal protrusion cannot be dated by imaging alone.
3. No acute fracture, marrow edema, or ligamentous injury.

Reported by: Anand Ramchandran, MD | Electronically signed 03/06/2024 15:03

HALLOWAY ORTHOPEDIC SPINE INSTITUTE

Spine Surgery and Non-Operative Spine Care

ORTHOPEDIC SPINE CONSULTATION

Patient	Whitfield, Marcus D. DOB 03/14/1987
Date of Service	03/14/2024
Provider	G. Halloway, MD, Orthopedic Spine Surgery
Referral Source	L. Osei-Bonsu, MD

HISTORY

This is a 36-year-old right-hand-dominant male warehouse logistics coordinator referred for evaluation of persistent right lower extremity radicular pain following a rear-end motor vehicle collision on 02/11/2024. He was evaluated in the emergency department on the date of injury with negative radiographs. Symptoms have persisted for five weeks despite naproxen, gabapentin, methocarbamol, and six sessions of physical therapy.

He describes low back pain of 5/10 and right leg pain of 7/10, the leg pain now dominant. Pain radiates from the right buttock through the posterolateral thigh and calf into the dorsum of the right foot. He reports numbness over the dorsum of the right foot and a sense that the right foot 'catches' when he is tired. He denies left leg symptoms, denies bowel or bladder dysfunction, and denies saddle anesthesia.

He remains off work. He rates his function as markedly limited and reports inability to sit through a full meal or drive more than 20 minutes.

REVIEW OF PRIOR RECORDS

I have personally reviewed the ED records of 02/11/2024, the lumbar radiographs of the same date, and the MRI lumbar spine of 03/06/2024. I have also reviewed the cervical spine MRI dated 03/2024 which demonstrates no significant cord compression. Physical therapy notes reviewed.

EXAMINATION

Gait antalgic on the right. Heel walking on the right is weak; toe walking intact. Lumbar flexion to 50 degrees, extension to 15 degrees with reproduction of leg pain.

Motor: Right EHL 4/5. Right tibialis anterior 4+/5. Right gastrocnemius 5/5. Left lower extremity 5/5 throughout. Hip abduction symmetric.

Sensory: Diminished pinprick right L5 dermatome, dorsum of foot and lateral calf.

Reflexes: Patellar 2+ bilaterally. Achilles 2+ left, 1+ right.

Provocative: Right straight leg raise positive at 35 degrees reproducing leg pain. Contralateral SLR negative. FABER negative bilaterally. No hip joint pain with internal rotation.

ASSESSMENT

Right L5 radiculopathy secondary to an L4-L5 right paracentral disc protrusion, symptomatic and correlating with imaging. Underlying multilevel degenerative disc disease is present and is chronic; in my opinion the acute focal protrusion and the onset of radicular symptoms are temporally related to the collision of 02/11/2024, in a spine with pre-existing degenerative change that was asymptomatic prior to the event.

PLAN

1. Continue physical therapy, extend authorization by 4 weeks.
2. Referral to interventional pain management for right L5 transforaminal epidural steroid injection.
3. Continue gabapentin, titrate to 300 mg TID.
4. Repeat evaluation in 6 weeks. If radicular pain and EHL weakness persist despite injection, would discuss right L4-L5 microdiscectomy.
5. Work status: remains off work.

Electronically signed: Gregory Halloway, MD | 03/14/2024

MERIDIAN REHABILITATION & SPORTS THERAPY

Outpatient Physical Therapy | Daily Treatment Notes

PHYSICAL THERAPY PROGRESS NOTE

Patient	Whitfield, Marcus DOB 03/14/1987
Date of Service	04/09/2024
Visit Number	14 of 24 authorized
Therapist	K. Petrosyan, PT, DPT

SUBJECTIVE

Patient reports back pain improved to 4/10 but states right leg pain is unchanged at 7/10 and remains the dominant complaint. He reports the leg pain worsens after the treadmill portion of the session. He continues to report numbness on the dorsum of the right foot. Reports he has been approved for an injection and is awaiting scheduling.

OBJECTIVE

Lumbar flexion	65% of normal (from 45% at eval)
Lumbar extension	60% of normal (from 30% at eval)
SLR right	Positive at 45 degrees (from 40 degrees)
MMT R great toe extension	4/5, unchanged
Oswestry Disability Index	38% (from 48%)

TREATMENT PROVIDED

- Therapeutic exercise 30 min: prone press-up progression, quadruped stabilization, standing hip hinge with dowel
- Manual therapy 15 min: grade III central PA mobilization L3-L5, soft tissue mobilization right QL and multifidus
- Neuromuscular re-education 15 min: gait training with cue for symmetric stance time
- Modalities: moist heat 10 min prior to manual therapy

ASSESSMENT

Patient demonstrating measurable improvement in lumbar mobility and axial pain with corresponding ODI improvement of 10 points. However, radicular symptoms and L5 motor deficit are unchanged at 14 visits, suggesting therapy alone is unlikely to resolve the nerve root involvement. Findings communicated to referring provider.

PLAN

Continue current plan of care. Reduce treadmill duration. Recommend patient proceed with scheduled injection. Reassess at visit 18.

Electronically signed: Karine Petrosyan, PT, DPT | 04/09/2024 11:42

SUMMIT INTERVENTIONAL PAIN & SPINE

Ambulatory Procedure Suite | Fluoroscopy Room 2

PROCEDURE NOTE - TRANSFORAMINAL EPIDURAL STEROID INJECTION

Patient	Whitfield, Marcus D. DOB 03/14/1987
Date of Procedure	04/23/2024
Proceduralist	D. Farhadi, MD
Pre-procedure Diagnosis	Right L5 radiculopathy, L4-L5 disc protrusion
Post-procedure Diagnosis	Same
Procedure	Right L5-S1 transforaminal epidural steroid injection under fluoroscopic guidance (CPT 64483)
Anesthesia	Local, 1% lidocaine. No sedation.
Complications	None

INDICATION

Patient with imaging-confirmed right paracentral L4-L5 disc protrusion and clinically corroborated right L5 radiculopathy, refractory to eight weeks of oral medication and fourteen sessions of physical therapy. Risks, benefits, and alternatives including continued conservative care and surgical decompression were discussed. Written informed consent obtained.

TECHNIQUE

Patient placed prone on the fluoroscopy table with a bolster beneath the abdomen. The lumbosacral region was prepped with chlorhexidine and draped in sterile fashion. Under intermittent fluoroscopic guidance the target foramen was identified and the C-arm obliqued to visualize the 'Scotty dog' view. Skin and subcutaneous tissue infiltrated with 1% lidocaine.

A 22-gauge spinal needle was advanced under intermittent fluoroscopy to the safe triangle beneath the pedicle. Position confirmed on AP and lateral views. Negative aspiration for blood and cerebrospinal fluid. Omnipaque 180 contrast, 1 mL, injected demonstrating appropriate epidural and perineural spread along the nerve root without vascular uptake.

A solution of dexamethasone 10 mg with 1 mL of 0.25% bupivacaine was then injected slowly. The patient tolerated the injection well and reported his typical radicular pain reproduced on contrast injection, then relieved.

POST-PROCEDURE

Patient monitored in recovery for 30 minutes. Motor and sensory function intact bilaterally. Ambulated without difficulty. Discharged home in stable condition with a responsible adult driver.

Pre-procedure pain 7/10. Immediate post-procedure pain 2/10 (anesthetic phase).

PLAN

- Pain diary for 14 days.
- Resume physical therapy in 3 days.
- Follow up in 3 weeks. May consider a second injection if partial but incomplete relief.
- Return to Dr. Halloway if relief is inadequate to discuss surgical options.

Electronically signed: Darius Farhadi, MD | 04/23/2024 14:35

MERIDIAN REHABILITATION & SPORTS THERAPY

Outpatient Physical Therapy | Daily Treatment Notes

PHYSICAL THERAPY PROGRESS NOTE

Patient	Whitfield, Marcus DOB 03/14/1987
Date of Service	04/09/2024
Visit Number	14 of 24 authorized
Therapist	(not signed)

SUBJECTIVE

Patient reports back pain 4/10 and right leg pain 5/10, improved. States he is tolerating sitting better and was able to drive 30 minutes without stopping. Denies new numbness.

OBJECTIVE

Lumbar flexion	65% of normal
SLR right	Negative
MMT R great toe extension	5/5
Oswestry Disability Index	38%

TREATMENT PROVIDED

- Therapeutic exercise 30 min
- Manual therapy 15 min
- Neuromuscular re-education 15 min

ASSESSMENT

Patient progressing well toward goals. Radicular symptoms resolving.

PLAN

Continue plan of care. Reassess at visit 18. Consider discharge planning.

NOTE NOT ELECTRONICALLY SIGNED. Draft status at time of records production.

HALLOWAY ORTHOPEDIC SPINE INSTITUTE

Handwritten Clinic Note - Transcribed from Scanned Chart

ORTHOPEDIC FOLLOW-UP VISIT (SCANNED / PARTIALLY ILLEGIBLE)

Patient	Whitfield, M.
Date of Service	05/21/2024
Provider	G. Halloway, MD

TRANSCRIPTION OF HANDWRITTEN NOTE

"Pt 4 wks s/p R TFESI. Reports [illegible] relief x 10 days then return of leg sx. Currently leg 6/10, back 4/10. Numbness dorsum R foot persists."

"Exam: gait antalgic R. EHL 4/5 R [illegible]. SLR + R at 35. Achilles 1+ R, 2+ L. No [illegible] change L side."

"A: R L5 radic, L4-5 HNP, failed conservative care incl PT x [illegible] visits, TFESI x1, meds."

"P: Discussed R L4-5 microdiscectomy. Risks incl bleeding, infection, dural tear, recurrent HNP, [illegible], no guarantee of relief. Pt wishes to proceed. Pre-op labs + [illegible] clearance. Sched OR."

"Work status: remains off work. Recheck post-op."

Signature illegible. Date stamp on scanned page: 05/21/2024. Transcribed by records department 06/14/2024.

RECORDS DEPARTMENT ANNOTATION

The original of this page was produced as a photocopy of a handwritten chart note of poor quality. Portions could not be read with confidence and are marked [illegible]. A request for a legible or typed duplicate was submitted to the provider on 06/14/2024. No response received as of the date of this production.

NORTHSTATE NEUROLOGY & ELECTRODIAGNOSTICS

Electromyography and Nerve Conduction Laboratory

EMG / NERVE CONDUCTION STUDY REPORT

Patient	Whitfield, Marcus D. DOB 03/14/1987
Date of Study	08/28/2024
Electromyographer	H. Vasquez-Lund, MD
Referring	G. Halloway, MD

CLINICAL INDICATION

Persistent lower extremity radicular symptoms following motor vehicle collision 02/11/2024. Evaluate for lumbosacral radiculopathy prior to surgical intervention.

NERVE CONDUCTION STUDIES

Peroneal motor, right	Distal latency 4.4 ms, amplitude 3.8 mV, CV 44 m/s
Peroneal motor, left	Distal latency 4.1 ms, amplitude 5.6 mV, CV 47 m/s
Tibial motor, bilateral	Within normal limits
Sural sensory, bilateral	Within normal limits, amplitudes symmetric
H-reflex	Right 31.2 ms, Left 30.8 ms - within normal limits

NEEDLE ELECTROMYOGRAPHY

Left tibialis anterior	Positive sharp waves 1+, reduced recruitment
Left extensor hallucis longus	Fibrillations 1+, large polyphasic MUAPs
Left gluteus medius	Positive sharp waves 1+
Left lumbar paraspinals L5	Fibrillations 1+
Left gastrocnemius	Normal
Left vastus lateralis	Normal
Right lower extremity screen	Normal insertional activity, no spontaneous activity, normal recruitment

IMPRESSION

1. Electrodiagnostic evidence of a chronic LEFT L5 radiculopathy with active denervation.
2. No electrodiagnostic evidence of peripheral polyneuropathy, plexopathy, or entrapment neuropathy.
3. Findings should be correlated with clinical examination and imaging.

Electronically signed: Helena Vasquez-Lund, MD | 08/28/2024

PIEDMONT COUNTY GENERAL HOSPITAL

Department of Surgery | Operating Room 6

OPERATIVE REPORT

Patient	Whitfield, Marcus D. MRN PCG-4471902
Date of Surgery	09/10/2024
Surgeon	G. Halloway, MD
Assistant	R. Nkemdirim, PA-C
Anesthesia	General endotracheal, administered by V. Sokolova, MD
Estimated Blood Loss	25 mL
Specimens	Disc material, sent to pathology
Complications	None

PREOPERATIVE DIAGNOSIS

Right L4-L5 herniated nucleus pulposus with right L5 radiculopathy, failed conservative management.

POSTOPERATIVE DIAGNOSIS

Same, confirmed intraoperatively.

PROCEDURE PERFORMED

Right L4-L5 hemilaminotomy with microdiscectomy and decompression of the right L5 nerve root (CPT 63030).

INDICATIONS

The patient is a 37-year-old male with a right L4-L5 disc protrusion causing right L5 radiculopathy with motor deficit, which has failed a course of physical therapy, oral neuropathic and anti-inflammatory medication, and one transforaminal epidural steroid injection. Symptoms have persisted for approximately seven months. Risks, benefits, and alternatives were reviewed and informed consent obtained.

DESCRIPTION OF PROCEDURE

The patient was brought to the operating room and general anesthesia was induced without difficulty. He was turned prone onto a Wilson frame with all pressure points padded. A surgical time-out was performed confirming patient identity, procedure, and correct operative level and laterality.

The lumbar region was prepped and draped in the usual sterile fashion. A localizing radiograph confirmed the L4-L5 interspace. A midline incision was made and carried through subcutaneous tissue. Subperiosteal dissection was carried out on the right side exposing the L4 lamina and L4-L5 interlaminar space. A retractor was placed and a repeat radiograph confirmed the level.

Using the operating microscope, a hemilaminotomy of the inferior margin of the right L4 lamina was performed with a high-speed burr and Kerrison rongeurs. The ligamentum flavum was incised and removed. The thecal sac and traversing right L5 nerve root were identified. The nerve root was noted to be tented and erythematous over a firm extruded disc fragment in the right lateral recess.

The nerve root was gently retracted medially. The fragment was delivered with a pituitary rongeur and additional loose degenerative nucleus pulposus was removed from the disc space. The lateral recess and foramen were then probed and found to be widely decompressed. The nerve root was noted to be freely mobile.

Hemostasis was obtained with bipolar cautery and thrombin-soaked Gelfoam. The wound was irrigated with antibiotic solution. The fascia was closed with 0-Vicryl, subcutaneous tissue with 2-0 Vicryl, and skin with a running subcuticular 4-0 Monocryl. A sterile dressing was applied. All counts were correct. The patient was awakened, extubated, and transferred to the post-anesthesia care unit in stable condition.

Electronically signed: Gregory Halloway, MD | 09/10/2024 13:48

ADDENDUM

Pathology returned as degenerative fibrocartilage consistent with intervertebral disc material. No malignancy.

Addendum not signed. No date recorded.

MERIDIAN REHABILITATION & SPORTS THERAPY

Outpatient Physical Therapy

PHYSICAL THERAPY DISCHARGE SUMMARY

Patient	Whitmore, Marcus
DOB	09/22/1979
MRN	MER-778213
Date of Discharge	04/30/2024
Referring Provider	B. Castellanos, MD
Body Region Treated	LEFT SHOULDER - rotator cuff repair, post-operative

REASON FOR REFERRAL

Status post left rotator cuff repair 01/18/2024. Post-operative rehabilitation protocol, phase III.

DISCHARGE STATUS

Left shoulder flexion	165 degrees (from 90 at initial evaluation)
Left shoulder abduction	160 degrees
Left external rotation	70 degrees
MMT supraspinatus	4+/5
DASH score	18% (from 62%)
Pain	1/10 at rest, 3/10 with overhead activity

GOALS STATUS

- Return to overhead reaching without pain: MET
- Lift 20 lb to shoulder height: MET
- Sleep on affected side: PARTIALLY MET
- Return to recreational golf: NOT MET, deferred to home program

ASSESSMENT

Patient has met the majority of stated goals and has plateaued with skilled intervention. Independent with home exercise program and demonstrated correct technique on all prescribed exercises.

PLAN

Discharge from skilled physical therapy. Continue home exercise program 4x weekly. Follow up with Dr. Castellanos as scheduled 05/14/2024.

Electronically signed: J. Mwangi, PT | 04/30/2024

HALLOWAY ORTHOPEDIC SPINE INSTITUTE
Post-Operative Follow-Up

POST-OPERATIVE OFFICE VISIT NOTES (TWO VISITS)

VISIT 1 - 09/24/2024 (two weeks post-operative)

Provider	R. Nkemdirim, PA-C
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Patient returns two weeks following right L4-L5 microdiscectomy. He reports immediate and near-complete resolution of the right leg pain upon waking from surgery. Current leg pain 1/10, incisional back pain 3/10. Numbness on the dorsum of the right foot has improved but is not fully resolved.

Examination: Incision clean, dry, intact, well-approximated without erythema or drainage. Sutures absorbable, no removal required. Motor: right EHL now 4+/5, improved from 4/5. Right ankle dorsiflexion 5/5. Sensation improved over the right L5 dermatome, diminished but present. SLR negative bilaterally. Gait non-antalgic.

Assessment: Excellent early post-operative course following right L4-L5 microdiscectomy. Motor improvement documented.

Plan: Begin post-operative physical therapy at 4 weeks. No lifting over 10 pounds, no bending or twisting for 6 weeks. Remains off work. Return in 6 weeks.

Electronically signed: Rita Nkemdirim, PA-C | 09/24/2024

VISIT 2 - 11/05/2024 (eight weeks post-operative)

Provider	G. Halloway, MD
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Patient is now eight weeks following right L4-L5 microdiscectomy. He reports right leg pain is essentially resolved, rated 0-1/10 and present only after prolonged standing. Low back pain 2-3/10, activity related. Residual patchy numbness over the dorsum of the right foot which he describes as no longer bothersome. He has completed eight post-operative physical therapy sessions.

Examination: Well-healed incision. Motor 5/5 throughout including right EHL. Sensation mildly diminished right L5 dermatome. SLR negative. Lumbar flexion 85% of normal. Gait normal. Able to heel and toe walk.

Assessment: Excellent outcome following right L4-L5 microdiscectomy with resolution of radicular pain and recovery of motor strength. Residual mild sensory deficit, which may or may not fully resolve.

Plan: Release to modified duty effective 11/11/2024 with a 25 pound lifting restriction for 8 weeks, then reassess for full duty. Complete remaining physical therapy. Continue home exercise program. Return in 3 months or as needed. Anticipate maximum medical improvement at approximately 6 months post-operative.

Electronically signed: Gregory Halloway, MD | 11/05/2024

RIVERBEND URGENT CARE - EAST
PRIOR RECORDS OBTAINED BY SUBPOENA | Produced 06/2024

URGENT CARE VISIT NOTE (HISTORICAL RECORD)

Patient	Whitfield, Marcus D. DOB 03/14/1987
Date of Service	08/17/2019
Provider	P. Lindqvist, PA-C

CHIEF COMPLAINT

Low back pain x 3 weeks.

HISTORY

32-year-old male presents with a three-week history of low back pain that began after moving furniture. Pain is described as an ache across the lower back, 5/10, worse with bending and prolonged standing. He reports intermittent radiation into the right buttock but denies pain below the knee. Denies numbness, weakness, bowel or bladder change, fever, or weight loss. Has tried over-the-counter ibuprofen with partial relief.

EXAMINATION

Lumbar paraspinal tenderness bilaterally. Flexion mildly limited by pain. Straight leg raise negative bilaterally. Strength 5/5 throughout. Sensation intact. Reflexes symmetric. Gait normal.

ASSESSMENT

Mechanical low back pain, subacute, with non-radicular right buttock referral. No red flags.

PLAN

- Naproxen 500 mg BID x 10 days
- Cyclobenzaprine 5 mg at bedtime PRN
- Home exercise handout provided
- Activity modification, avoid heavy lifting x 2 weeks
- Follow up with primary care if not improved in 3 weeks or if new radiating pain, numbness, or weakness develops

Electronically signed: P. Lindqvist, PA-C | 08/17/2019

ATTACHED: RADIOLOGY REPORT

LUMBAR SPINE RADIOGRAPHS, 2 VIEWS - 08/17/2019

FINDINGS: Vertebral body heights preserved. Mild disc space narrowing at L4-L5. Early facet arthropathy at L4-L5 and L5-S1. No fracture or listhesis.

IMPRESSION: Early degenerative disc disease and facet arthropathy at L4-L5 and L5-S1. No acute finding.

Reported by: M. Iverson, MD | 08/17/2019

GREENLEAF COMMUNITY PHARMACY #218

Patient Medication History Printout | Generated 12/02/2024

PHARMACY DISPENSING RECORD

Patient	Whitfield, Marcus D. DOB 03/14/1987
Allergies on File	NONE DOCUMENTED
Insurance	Meridian Health Plan, Member 88-2210-447

DISPENSING HISTORY

02/11/2024	Naproxen 500 mg tab, qty 30, Rx by Nakamura S MD, filled 22:40
02/11/2024	Methocarbamol 750 mg tab, qty 30, Rx by Nakamura S MD, filled 22:40
02/11/2024	Acetaminophen/Codeine 300-30 mg tab, qty 12, Rx by Nakamura S MD, filled 22:41
02/19/2024	Gabapentin 300 mg cap, qty 30, Rx by Osei-Bonsu L MD
03/18/2024	Gabapentin 300 mg cap, qty 90, Rx by Osei-Bonsu L MD, titration to TID
03/18/2024	Naproxen 500 mg tab, qty 60, refill, Rx by Osei-Bonsu L MD
04/23/2024	Ondansetron 4 mg ODT, qty 10, Rx by Farhadi D MD
06/03/2024	Gabapentin 300 mg cap, qty 90, refill
09/10/2024	Oxycodone/Acetaminophen 5-325 mg tab, qty 20, Rx by Halloway G MD, post-operative
09/10/2024	Docusate sodium 100 mg cap, qty 30, Rx by Halloway G MD
09/10/2024	Methocarbamol 750 mg tab, qty 40, Rx by Halloway G MD
09/22/2024	Oxycodone/Acetaminophen 5-325 mg tab, qty 12, refill x1, Rx by Halloway G MD
10/28/2024	Gabapentin 300 mg cap, qty 90, refill

PHARMACIST NOTES

02/11/2024 - Counseled on drowsiness with methocarbamol and codeine combination. Advised no driving.

09/10/2024 - Opioid counseling provided. Naloxone offered and declined. PDMP reviewed, no concerning findings.

This printout reflects prescriptions filled at Greenleaf Community Pharmacy locations only and may not represent a complete medication history.

MERIDIAN HEALTH SYSTEM - PATIENT FINANCIAL SERVICES

Itemized Statement of Charges | Claim PI-2024-8871

ITEMIZED BILLING SUMMARY

Patient	Whitfield, Marcus D. Account ED-24-0211-556
Statement Period	02/11/2024 through 11/30/2024
Statement Generated	12/09/2024

ITEMIZED CHARGES

02/11/2024	EMS ground transport, ALS-1 (A0427)	\$1,480.00
02/11/2024	ED visit, level 4 (99284)	\$2,310.00
02/11/2024	CT cervical spine w/o contrast (72125)	\$1,875.00
02/11/2024	Radiographs lumbar spine 3 views (72100)	\$ 410.00
02/11/2024	Pharmacy, ED administered	\$ 186.00
02/19/2024	Office visit, established, level 3 (99213)	\$ 245.00
02/26/2024	PT initial evaluation, moderate complexity (97162)	\$ 310.00
03/06/2024	MRI lumbar spine w/o contrast (72148)	\$3,240.00
03/12/2024	MRI cervical spine w/o contrast (72141)	\$3,180.00
03/14/2024	Consultation, spine surgery (99244)	\$ 595.00
02/26-05/07/2024	Physical therapy, 22 visits, bundled	\$4,840.00
04/23/2024	TFESI lumbar, single level (64483)	\$2,650.00
04/23/2024	Fluoroscopic guidance (77003)	\$ 420.00
06/11/2024	TFESI lumbar, single level (64483)	\$2,650.00
08/28/2024	EMG/NCS, lower extremity (95886, 95908)	\$1,290.00
09/10/2024	Laminotomy w/ discectomy, 1 interspace (63030)	\$8,950.00
09/10/2024	Anesthesia, spine surgery	\$1,720.00
09/10/2024	Facility, ambulatory surgery	\$6,400.00
09/24/2024	Post-op visit, global period	\$ 0.00
10/2024	Post-operative physical therapy, 8 visits	\$1,760.00
11/05/2024	Office visit, established, level 3 (99213)	\$ 245.00

SUMMARY

Total Charges	\$44,756.00
Insurance Payments	\$18,204.36
Adjustments / Contractual Write-offs	\$14,988.14
Patient Responsibility	\$11,563.50
Outstanding Balance	\$11,563.50

Charges reflect the master charge description master rate and may not reflect negotiated rates. Lien filed with Meridian Health System Recovery Unit 12/09/2024.

ATLAS LOGISTICS GROUP - HUMAN RESOURCES
Occupational Health & Work Status File

EMPLOYER WORK STATUS AND WAGE RECORD

Employee	Whitfield, Marcus D.
Employee ID	ALG-33417
Position	Warehouse Logistics Coordinator, Shift Lead
Date of Hire	06/02/2019
Rate of Pay	\$28.40 per hour, 40 hours regular plus average 6 hours overtime weekly

WORK STATUS CHRONOLOGY

02/12/2024	Off work per ED note dated 02/11/2024
02/19/2024	Light duty offered by employer; declined as unavailable in role
02/19/2024	Off work through 03/04/2024 per Dr. Osei-Bonsu
03/14/2024	Remains off work per Dr. Halloway
05/21/2024	Remains off work per Dr. Halloway
09/10/2024	Surgical leave commences
11/05/2024	Released to modified duty effective 11/11/2024, 25 lb restriction
11/11/2024	Returned to work, modified duty, 32 hours per week
01/06/2025	Returned to full duty per employer records

WAGE LOSS CALCULATION (employer-provided)

Total days off work	02/12/2024 through 11/10/2024
Regular hours lost	Approximately 1,560 hours
Overtime hours lost	Approximately 234 hours
Partial wage loss, modified duty period	11/11/2024 through 01/05/2025, 8 hours per week reduction

HR ANNOTATION

Employee was in good standing at the time of leave with no active disciplinary matters. Short-term disability benefits were paid at 60% of base wage from 02/26/2024 through 08/26/2024, the maximum benefit period. No benefits were payable thereafter. Employee used 42 hours of accrued paid time off prior to short-term disability commencement.

Prepared by: Danica Brant, HR Business Partner | 01/14/2025

OFFICE OF THE RECORDS CUSTODIAN
Certification and Production Log

RECORDS PRODUCTION CERTIFICATION

Matter	Whitfield v. Corrigan Hauling LLC
Claim Number	PI-2024-8871
Records Requested By	Requesting Party (records request dated 12/18/2024)
Date of Production	01/22/2025
Total Pages Produced	22

CERTIFICATION

The undersigned certifies that the accompanying documents are true and correct copies of records maintained in the ordinary course of business, produced in response to the request referenced above, and that the records were made at or near the time of the events described by persons with knowledge, or from information transmitted by such persons.

PRODUCTION LOG - SOURCES

Piedmont County EMS	1 page
Piedmont County General Hospital - ED	3 pages
Piedmont County General Hospital - Radiology	1 page
Piedmont County General Hospital - Surgery	1 page
Northgate Family Medicine	1 page
Meridian Rehabilitation & Sports Therapy	4 pages
Valley Imaging Partners	1 page
Halloway Orthopedic Spine Institute	3 pages
Summit Interventional Pain & Spine	1 page
Northstate Neurology & Electrodiagnostics	1 page
Riverbend Urgent Care (historical, subpoenaed)	1 page
Greenleaf Community Pharmacy	1 page
Meridian Health System - Financial Services	1 page
Atlas Logistics Group - HR	1 page
This certification	1 page

ITEMS REQUESTED BUT NOT PRODUCED

- MRI cervical spine, referenced in orthopedic consultation of 03/14/2024 and billed 03/12/2024. Report not located in the requested file. Follow-up request pending.
- Legible duplicate of the handwritten orthopedic note of 05/21/2024. Requested 06/14/2024, no response received.
- Clinical documentation corresponding to the transforaminal epidural steroid injection billed on 06/11/2024. Not located.
- Physical therapy daily notes for visits 15 through 22. Not located in this production.

Certified by: Office of the Records Custodian | 01/22/2025